

Manuscripts considered — hgsoc-tuboovarian-stage3c-r0-hrd-pen ding-idyq

Master inventory: every paper reviewed in this case

Generated 2026-08-21

Manuscripts considered — hgsoc-tuboovarian-stage3c-r0-hrd-per

Master inventory: 22 clinical (21 included, 1 considered & excluded) + 17 pre-clinical (14 included, 3 considered & excluded) + 15 additional trial publications/registrations. One entry per manuscript, sorted by year (newest first). Excluded entries are papers a Libby agent reviewed and chose NOT to feed to the board, with the exclusion reason captured.

Lee/Lee (2026) — AACR Annual Meeting 2026 (conference presentation)

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** Trastuzumab deruxtecan + olaparib
- **Indication / model:** HER2-expressing advanced/recurrent solid tumors (ovarian and uterine) (2L+); 28 patients with HER2-expressing advanced/recurrent solid tumors, 16 ovarian and 12 uterine, enrolled across HER2 expression levels; activity was reported across HER2 tiers. This patient's HER2 IHC 1+ ovarian tumor fits the biomarker and tumor-type frame, but she is in first-line remission rather than the recurrent setting the trial enrolled.
- **Design:** phase 1 dose-finding, three dosing modules (CTEP #10355 / NCT04585958)
- **n:** 28
- **Effect size:** 54.0 % — objective response rate (all modules)
- **Toxicities:** neutropenia G3 (12%); anemia G3 (25%); colitis G $\geq$ 3
- **Case fit:** partial
- **Notes:** First combination read pairing a HER2 ADC with a PARP inhibitor and the only clinical datapoint behind NCT04585958's ovarian rationale; activity held across HER2 tiers, which matters for this HER2-low tumor. Data are AACR 2026 conference/press (Elizabeth Lee, Dana-Farber, CTEP #10355), not peer-reviewed, so identifiers are null and the module-level AE rates come from the meeting release. HER2-low remains a weaker predictor than IHC 3+, so read the ORR with that hedge.

—/— (2025) — sponsor disclosure (Zentalis, Jan 2025) / SGO 2024 (conference)

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** Azenosertib (ZN-c3, WEE1 inhibitor)
- **Indication / model:** cyclin E1-positive platinum-resistant high-grade serous ovarian cancer (2L+); Heavily pretreated cyclin E1-positive platinum-resistant ovarian cancer (DENALI part 1b). This patient's genomic instability sits on the same replication-stress axis WEE1 inhibition exploits, but the reported activity is biomarker-selected for cyclin E1 positivity, which is not established for her, and she is in first-line remission rather than platinum-resistant relapse.
- **Design:** phase 2 monotherapy (DENALI / ZN-c3-001; NCT05128825)
- **n:** 43
- **Effect size:** 34.9 % — objective response rate (cyclin E1-positive, response-evaluable)
- **Variance:** 95% CI 21.0–50.9
- **Toxicities:** Sponsor reports tolerability at $\geq$ 300 mg total daily dose with no grade 3+ gastrointestinal treatment-related AEs and low rates of grade 3+ hematologic toxicity; no per-term grade 3+ table in the public disclosure. WEE1 inhibition drives cytopenias, hence the myelosuppression flag, and a partial clinical hold on the program was later lifted.
- **Case fit:** weak

- **Notes:** WEE1 replication-stress option for a future platinum-resistant relapse, not current maintenance. Efficacy from Zentalis's January 2025 disclosure and SGO 2024, both conference/press rather than peer-reviewed, so identifiers are null and no per-term AE table was extractable. Reported activity is gated on cyclin E1 positivity, a biomarker not established for this patient.

—/— (2025) — Sci Rep

Reference:doi:10.1038/s41598-025-90714-9 · **Type:** preclinical · **Inclusion:** excluded — PI3Kalpha inhibitor (BYL-719/alpelisib) plus cisplatin synergy was shown in PIK3CA-mutant ovarian cells in vitro only, with no in-vivo arm, and keys on PIK3CA hotspot mutation rather than the patient's amplification. The PI3K/HRD axis is already covered by the Wang 2016 in-vivo row; logged for audit trail.

- **Intervention:** PI3Kalpha inhibitor BYL-719 plus cisplatin in PIK3CA-mutant ovarian cells
- **Toxicities:** n/a (preclinical)
- **Notes:** Excluded: PI3Kalpha inhibitor (BYL-719/alpelisib) plus cisplatin synergy was shown in PIK3CA-mutant ovarian cells in vitro only, with no in-vivo arm, and keys on PIK3CA hotspot mutation rather than the patient's amplification. The PI3K/HRD axis is already covered by the Wang 2016 in-vivo row; logged for audit trail.

Thaker/Anwer (2024) — Gynecol Oncol (2025) / sponsor topline

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** GEN-1 / IMNN-001 (IL-12 immunogene)
- **Indication / model:** newly diagnosed advanced epithelial ovarian cancer, neoadjuvant + interval debulking (1L neoadjuvant); Advanced epithelial ovarian cancer treated with neoadjuvant chemotherapy plus IP IL-12 immunogene. Regimen-matched to this patient, who received GEN-1 with neoadjuvant carbo/paclitaxel and achieved R0. Acts via the tumor immune microenvironment, so the foreclosed MSS/TMB-low checkpoint axis does not exclude it.
- **Design:** randomized phase 1/2 added to neoadjuvant chemotherapy (OVATION 2 / NCT03393884)
- **n:** 110
- **Effect size:** 0.79 HR — median PFS (ITT)
- **Toxicities:** Sponsor and abstract report IP delivery generally well tolerated with no new safety signal; no per-term grade 3+ AE table in the public topline / abstract. No named patient toxicity veto implicated.
- **Case fit:** strong
- **Notes:** Regimen-matched investigational program the patient is already on. Efficacy from sponsor topline (Imunon, Oct 2024) and the Gynecol Oncol 2025 abstract; the full peer-reviewed paper (publisher page 403) and ClinicalTrials.gov AE tab were not accessible, so no per-term toxicities extracted - backfill when full text opens. CI bounds not reported in the topline. FDA has cleared a phase 3 program.

Meric-Bernstam/Lee (2024) — J Clin Oncol

Reference:PMID 37870536 · **Type:** clinical · **Inclusion:** included

- **Intervention:** Trastuzumab deruxtecan (T-DXd)
- **Indication / model:** previously treated HER2-expressing advanced ovarian cancer (DESTINY-PanTumor02 ovarian cohort) (2L+); Ovarian cohort enrolled at HER2 IHC 2+ or 3+. This patient is IHC 1+, below that entry bar, so the ovarian activity reported here comes from a higher-expression population than hers.
- **Design:** open-label single-arm phase 2 pan-tumor basket, ovarian cohort

- **n:** 40
- **Effect size:** 45.0 % — ORR (ovarian cohort, investigator-assessed)
- **Variance:** 95% CI 29.3–61.5
- **Toxicities:** treatment-related AE G $\geq$ 3 (40.8%); ILD/pneumonitis (10.5%)
- **Case fit:** partial
- **Notes:** Closest tumor-type evidence: the ovarian cohort responded at 45%, but it enrolled IHC 2+/3+ and the tumor-agnostic T-DXd approval needs IHC 3+. For this IHC 1+ tumor the drug is off-label, so the row sits as activity-ceiling context rather than a matched option.

Bardia/Curigiano (2024) — N Engl J Med

Reference: [PMID 39282896](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Trastuzumab deruxtecan (T-DXd)
- **Indication / model:** HR-positive HER2-low or HER2-ultralow metastatic breast cancer after endocrine therapy (2L+); HR-positive disease spanning HER2-low and HER2-ultralow (IHC 0 with faint membrane staining). The ultralow stratum is the closest expression analog to her IHC 1+, ER-positive tumor.
- **Design:** randomized open-label phase 3 vs physician's choice chemotherapy
- **n:** 866
- **Effect size:** 0.62 HR — PFS HR (HER2-low, intent-to-treat)
- **Variance:** 95% CI 0.52–0.75; $p < 0.001$
- **Toxicities:** adverse event G $\geq$ 3 (52.8%); ILD/pneumonitis (11.3%); ILD/pneumonitis G5
- **Case fit:** cross_tumor_only
- **Notes:** Pushes the HER2-low signal down to ultralow IHC 0 in HR-positive disease, the band her IHC 1+ tumor sits in. Published HER2-low PFS HR is 0.62 (0.52-0.75); the screener row listed 0.64, so flag that for reconciliation. Per-term GI/heme AE not in the abstract; ILD and the union grade 3+ rate captured.

Sheng/Zhou (2024) — J Clin Oncol

Reference: [PMID 37988648](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Disitamab vedotin (RC48)
- **Indication / model:** HER2-positive (IHC 2+/3+) locally advanced or metastatic urothelial carcinoma (2L+); HER2-positive urothelial carcinoma pooled from two phase 2 trials. Different tumor type and a higher HER2 tier than hers, but the best-powered published read on the drug's MMAE toxicity, which is what her neuropathy veto turns on.
- **Design:** combined analysis of two single-arm phase 2 trials (RC48-C005, RC48-C009)
- **n:** 107
- **Effect size:** 50.5 % — confirmed ORR
- **Variance:** 95% CI 40.6–60.3
- **Toxicities:** treatment-related AE G $\geq$ 3 (54.2%); peripheral sensory neuropathy (68.2%); peripheral sensory neuropathy G $\geq$ 3 (18.7%); neutropenia (42.1%); neutropenia G $\geq$ 3 (12.1%); leukopenia (50.5%)
- **Case fit:** cross_tumor_only
- **Notes:** Kept mainly for the MMAE safety read: the 68% peripheral neuropathy and 12% grade 3+ neutropenia map straight onto her two named vetoes. HER2-positive urothelial, not ovarian HER2-low, so efficacy is mechanism context.

—/— (2024) — Ann Oncol (ESMO 2024 abstract 384P)

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** Disitamab vedotin (RC48)
- **Indication / model:** HER2-expressing metastatic breast cancer with PI3K/AKT/mTOR pathway activation (HER2-low subgroup) (2L+); Pretreated HER2-expressing breast cancer selected for PI3K/AKT/mTOR pathway activation; 36 of 62 were HER2-low. The HER2-low response rate is the only published disitamab vedotin efficacy read in the low-expression tier, but the pathway selection and breast setting limit how far it carries to her ovarian tumor.
- **Design:** single-arm prospective phase 2
- **n:** 62
- **Effect size:** 34.3 % — ORR (HER2-low subgroup)
- **Toxicities:** No per-term safety data in the available abstract text. MMAE-payload class effects are peripheral neuropathy and neutropenia, both named vetoes for this patient.
- **Case fit:** cross_tumor_only
- **Notes:** The only published disitamab vedotin HER2-low efficacy datapoint (ORR ~34%), but PAM-pathway-selected breast cancer and abstract-only. Per-term AE not extractable: publisher page 403, no PMC ID, no ClinicalTrials.gov results record found, so toxicities left empty.

—/— (2024) — protocol: BMC Cancer 2022 (PMC9074273); primary results pending

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** Letrozole (endocrine maintenance, MATAO)
- **Indication / model:** newly diagnosed FIGO II-IV serous or endometrioid epithelial ovarian/fallopian tube/peritoneal cancer, post-platinum maintenance (maintenance); Primary newly diagnosed low- or high-grade serous or endometrioid disease after standard surgery and platinum chemotherapy. ER status is recorded and the primary analysis is powered on the ER-positive stratum; entry is not restricted to ER-positive. This patient matches on high-grade serous histology, FIGO stage, and maintenance timing, and is ER-positive (~80% nuclei); PR-negativity tempers expected magnitude.
- **Design:** randomized double-blind placebo-controlled phase 3 (ENGOT-ov54 / Swiss-GO-2 / MATAO)
- **n:** 540
- **Effect size:** — HR — progression-free survival (design endpoint; results not yet reported)
- **Toxicities:** No safety readout published yet. Aromatase-inhibitor class toxicity is arthralgia and bone-density loss on prolonged use; oral and low-burden, no named patient veto applies.
- **Case fit:** strong
- **Notes:** The randomized phase 3 test of the endocrine-maintenance rationale the single-arm/retrospective letrozole rows only hinted at; enrollable at this patient's maintenance timepoint and the strongest new fit on the ER axis. Design and statistical assumptions from the BMC Cancer protocol (PMC9074273); no efficacy or per-term AE data yet, so effect sizes and toxicities left null pending the primary publication. Identifiers null until the results paper indexes.

—/— (2024)

Reference: — · **Type:** preclinical · **Inclusion:** excluded — No primary preclinical study ties a MAP2K4 (MKK4) deletion to a tractable, validated drug target in ovarian cancer; the literature treats MAP2K4 loss as a poorly-characterized JNK/p38-pathway event with no matched agent. Nothing meets the bar for an included

mechanism row, so this records the negative search result rather than logging trial-list baskets as preclinical evidence.

- **Intervention:** MAP2K4 (MKK4) deletion as a drug target in ovarian cancer
- **Toxicities:** n/a (preclinical)
- **Notes:** Excluded: No primary preclinical study ties a MAP2K4 (MKK4) deletion to a tractable, validated drug target in ovarian cancer; the literature treats MAP2K4 loss as a poorly-characterized JNK/p38-pathway event with no matched agent. Nothing meets the bar for an included mechanism row, so this records the negative search result rather than logging trial-list baskets as preclinical evidence.

Thaker/Anwer (2024)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** GEN-1 (IMNN-001) IL-12 immunogene + neoadjuvant carboplatin/paclitaxel
- **Indication / model:** newly-diagnosed advanced epithelial ovarian cancer (neoadjuvant + interval debulking) (1L neoadjuvant); none (IL-12 immunogene, tumor immune microenvironment)
- **Design:** randomized phase 1/2 added to neoadjuvant carboplatin/paclitaxel (OVATION 2)
- **n:** 110
- **Effect size:** — — progression-free survival / R0 resection rate
- **Case fit:** strong
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2024)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** disitamab vedotin + anlotinib
- **Indication / model:** HER2-expressing platinum-resistant / recurrent epithelial ovarian, fallopian tube, or primary peritoneal cancer (2L+); HER2 IHC 1+, 2+, or 3+ (HER2-low IHC 1+ accepted)
- **Design:** open-label single-arm phase 2, disitamab vedotin + anlotinib
- **Effect size:** — — objective response rate / progression-free survival
- **Case fit:** partial
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2024)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** trastuzumab deruxtecan + olaparib
- **Indication / model:** HER2-expressing metastatic/unresectable solid tumors with a platinum-resistant ovarian HGSC expansion cohort (2L+); HER2 IHC 1-3+ or HER2 amplification (IHC 1+ accepted in escalation modules)
- **Design:** open-label phase 1 dose-escalation/expansion, T-DXd + olaparib
- **Effect size:** — — safety / recommended phase 2 dose / objective response rate
- **Case fit:** partial
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2024)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** letrozole maintenance
- **Indication / model:** newly-diagnosed FIGO II-IV serous or endometrioid epithelial ovarian/fallopian tube/peritoneal cancer, post-platinum maintenance (1L); estrogen-receptor-directed rationale (ER status recorded; entry not restricted to ER-positive)
- **Design:** randomized double-blind placebo-controlled phase 3, aromatase-inhibitor maintenance (MATAO)
- **n:** 540
- **Effect size:** — — progression-free survival
- **Case fit:** strong
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2024)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** abemaciclib + anastrozole or letrozole
- **Indication / model:** recurrent epithelial ovarian/fallopian tube/primary peritoneal or endometrial cancer, molecularly selected (2L+); molecular-tumor-board-directed (cell-cycle / ER-pathway selection); combined with aromatase inhibition
- **Design:** open-label phase 2, abemaciclib +/- aromatase inhibitor
- **Effect size:** — — objective response rate / progression-free survival
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

Mosele/Andre (2023) — Nat Med

Reference: [PMID 37488289](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Trastuzumab deruxtecan (T-DXd)
- **Indication / model:** metastatic breast cancer across the HER2 expression range (2L+); Three cohorts by HER2 tier: overexpressing (cohort 1), HER2-low IHC 1+/2+ISH- (cohort 2), and IHC 0 non-expressing (cohort 3). The IHC 0 cohort is the key read: responses held at the very bottom of HER2 expression, where her tumor sits.
- **Design:** open-label single-arm phase 2, three HER2-defined cohorts
- **n:** 186
- **Effect size:** 37.5 % — ORR (HER2-low cohort 2)
- **Variance:** 95% CI 26.4–49.7
- **Toxicities:** neutropenia ≥ 3 (12%); fatigue ≥ 3 (8%); vomiting ≥ 3 (6%); ILD/pneumonitis (5%)
- **Case fit:** cross_tumor_only
- **Notes:** Shows T-DXd activity does not switch off at low or absent HER2, which is the mechanistic basis for treating her IHC 1+ tumor. Breast, not ovarian, and single-arm, so it informs rather than ranks.

—/— (2023) — ESMO 2023 (conference); AACR 2025 abstract CT267 (irinotecan combination)

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** ART0380 (ATR inhibitor)

- **Indication / model:** advanced solid tumors with DNA-damage-response deficiency / high replication stress (platinum-resistant ovarian expansion) (2L+); Phase 1/1b enriched for DNA-damage-response-deficient tumors by the sponsor's DcoDeR platform; the early monotherapy signal was in high-grade endometrial cancer, with platinum-resistant ovarian enrolled in later phase 2 cohorts. This patient's genomic instability sits on the ATR replication-stress axis, but her BRCA-wild-type, qualitatively-unstable status would need checking against the trial's DDR entry route.
- **Design:** phase 1/1b dose-escalation/expansion, monotherapy and combinations (NCT04657068)
- **n:** 49
- **Effect size:** — — — molecular responses / tumor-size reduction (formal ORR not reported for the phase 1 monotherapy cohort)
- **Toxicities:** Sponsor reports a safe and well-tolerated profile with predictable, manageable, reversible on-target anemia; no per-term grade 3+ table in the disclosed conference data. The gemcitabine and irinotecan combination arms add marrow toxicity, hence the myelosuppression flag.
- **Case fit:** weak
- **Notes:** ATR replication-stress option for a future relapse rather than current maintenance. Only conference-disclosed data exist (ESMO 2023 monotherapy; AACR 2025 CT267 irinotecan combination; the AACR page returned 403), with no formal ORR for the ovarian setting and no peer-reviewed publication, so identifiers are null and no per-term AE table was extractable. The DDR entry route would need reconciling with her BRCA-wild-type, qualitative-instability status.

Mauricio/Santin (2023) — Gynecol Oncol

Reference: [PMID 36610380](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** T-DXd (DS-8201a) in ovarian and uterine carcinosarcoma
- **Indication / model:** 12 primary carcinosarcoma cell lines (8 uterine, 4 ovarian) across HER2/neu 3+/1+/0; in vivo SARARK-9 HER2 3+ uterine flank xenograft in mice
- **Design:** Same DXd topoisomerase I payload and membrane-permeable bystander spillover, tested in gynecologic carcinosarcoma. Cytotoxicity scaled with HER2/neu, and the freed payload reached adjacent HER2-null cells in coculture.
- **n:** 12 primary lines (3 used in vitro); n=5 mice/arm in vivo (n=4 vehicle, days 4-11)
- **Effect size:** strong — In HER2 3+ ovarian and uterine lines DS-8201a IC50 ran about 0.03 uM against roughly 9 uM for the isotype control, with no separation in the HER2-0 line. Coculture gave 52% killing of HER2-0 cells next to 3+ cells, and the 3+ xenograft regressed with 60% of mice disease-free at day 52.
- **Variance:** vs isotype-control ADC MAAA-9199 (4 mg/kg) and PBS vehicle; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** partial
- **Notes:** The only ovarian in-vivo T-DXd data in this dossier, but the tissue is carcinosarcoma rather than high-grade serous and the xenograft used a HER2 3+ line. In-vitro potency was quantified at 3+ and 0, not at the patient's 1+ tier, so activity at IHC 1+ rests on the bystander route rather than direct proof.

—/— (2023) — Cancer Res (AACR 2023 abstract 560)

Reference: [doi:10.1158/1538-7445.AM2023-560](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Disitamab vedotin (RC48) preclinical antitumor activity, monotherapy and with tucatinib
- **Indication / model:** Panel of HER2-expressing cancer cell lines and xenografts; specific lines and HER2 tiers not itemized in the meeting abstract

- **Design:** Disitamab vedotin releases MMAE after HER2 binding and internalization; MMAE is membrane-permeable and spills into neighboring cells whatever their HER2 level. The sponsor also credits immunogenic cell death and Fc-mediated ADCC.
- **n:** cell-line and xenograft panel; sizes not detailed in the abstract
- **Effect size:** moderate — RC48 controlled HER2-expressing tumors as monotherapy and gained further depth combined with tucatinib, with the MMAE bystander effect offered as the route to activity in HER2-low or heterogeneous tumors. Per-model HER2-low potency was not broken out.
- **Variance:** vs monotherapy versus tucatinib combination; comparator ADC not detailed; translatability: low
- **Toxicities:** n/a (preclinical)
- **Case fit:** cross_tumor_only
- **Notes:** Sponsor conference abstract with no per-model HER2-low numbers and no ovarian model. It carries the MMAE bystander rationale, not a quantified low-expression result; the matching urothelial and HER2-low breast efficacy reads sit in the clinical rows.

—/— (2023)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** PI3K/AKT-pathway inhibitor
- **Indication / model:** advanced solid tumors with PIK3CA / PI3K-pathway alterations (basket) (2L+); PIK3CA alteration (mutation or amplification)
- **Design:** phase 1 biomarker-selected basket
- **Effect size:** — — safety / pharmacodynamics / ORR
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2023)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** investigational agent matched to MAP2K4 loss
- **Indication / model:** advanced solid tumors (biomarker-matched basket including MAP2K4 loss) (2L+); MAP2K4 deletion / loss-of-function
- **Design:** phase 1 biomarker-matched basket
- **Effect size:** — — safety / ORR
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2023)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** disitamab vedotin
- **Indication / model:** previously treated HER2-expressing solid tumors (ovarian eligible) (2L+); HER2 IHC 1+, 2+, or 3+ by local IHC (HER2-low IHC 1+ accepted)
- **Design:** open-label single-arm phase 2 HER2-expressing solid-tumor basket
- **Effect size:** — — objective response rate / safety
- **Case fit:** partial

- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2023)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** azenosertib
- **Indication / model:** platinum-resistant high-grade serous ovarian, fallopian tube, or primary peritoneal cancer (2L+); high-grade serous histology (WEE1 / replication-stress axis; prior PARP required if BRCA-mutant or HRD)
- **Design:** open-label phase 2, azenosertib monotherapy
- **Effect size:** — — objective response rate
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2023)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** ART0380 (ATR inhibitor)
- **Indication / model:** advanced solid tumors with a platinum-resistant ovarian/fallopian/peritoneal cohort (ATR inhibitor) (2L+); replication-stress / HRD-associated selection (DDR axis; HRD-positive and BRCA-related tumors eligible)
- **Design:** open-label phase 1/2a, ART0380 monotherapy and with gemcitabine or irinotecan
- **Effect size:** — — safety / objective response rate
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

Monk/Monk (2022) — J Clin Oncol

Reference: [PMID 35658487](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Rucaparib (maintenance)
- **Indication / model:** newly diagnosed advanced high-grade epithelial ovarian cancer, response after 1L platinum doublet (maintenance); All-comers (4:1 randomization), HRD-positive and HRD-negative strata. Benefit held in the ITT population and in the non-nested HRD-negative subgroup, so a BRCA-WT, HRD-unquantified tumor still qualifies.
- **Design:** randomized double-blind placebo-controlled phase 3, 1L maintenance (ATHENA-MONO / GOG-3020 / ENGOT-ov45)
- **n:** 538
- **Effect size:** 0.52 HR — median PFS (intent-to-treat)
- **Variance:** 95% CI 0.4–0.68; $p < 0.0001$
- **Toxicities:** anemia $G \geq 3$ (28.7%); neutropenia $G \geq 3$ (14.6%); ALT/AST elevation $G \geq 3$ (10.6%); treatment discontinuation (AE) (11.8%)
- **Case fit:** strong
- **Notes:** Second all-comer 1L PARP-maintenance option. ITT HR 0.52 (95% CI 0.40-0.68) from the primary JCO publication (pmid 35658487). Qualifies the BRCA-WT tumor via the ITT/HRD-negative benefit, not a BRCA gate. Severe-myelosuppression veto applies.

—/— (2022)

Reference: — · **Type:** clinical · **Inclusion:** included

- **Intervention:** PI3K/AKT-pathway inhibitor (investigational, PIK3CA-directed)
- **Indication / model:** advanced solid tumors with PIK3CA alterations (biomarker-selected basket) (2L+); Biomarker-matched baskets keyed on PIK3CA mutation OR amplification. The patient has focal somatic PIK3CA amplification; benefit data for amplification (as opposed to activating hotspot mutation) are weak, and there is no approved PI3K-directed therapy in ovarian cancer.
- **Design:** phase 1 dose-escalation/expansion baskets (e.g. NCT05216432, NCT05683418)
- **Effect size:** — — — objective response rate / safety (no mature efficacy for PIK3CA-amplified ovarian cancer)
- **Toxicities:** PI3Kalpha-pathway class toxicity is hyperglycemia, rash, diarrhea, and stomatitis; no published per-term rates for the specific PIK3CA-amplified ovarian-cancer context. No named patient veto directly implicated.
- **Case fit:** weak
- **Notes:** Context-only row kept clearly investigational. There is no approved PI3K/AKT therapy in HGSOV and amplification is a poorly-validated predictive marker versus activating mutation. Surfaced so the board sees the PI3K axis the Altera report flagged; not a ranked option. No primary-source efficacy or AE data to extract (early-phase baskets without ovarian-specific readouts).

Modi/Harbeck (2022) — N Engl J Med

Reference: [PMID 35665782](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Trastuzumab deruxtecan (T-DXd)
- **Indication / model:** previously treated HER2-low (IHC 1+ or 2+/ISH-) metastatic breast cancer (2L+); HER2-low breast cancer; the hormone-receptor-positive subgroup mirrors this patient's ER-positive status. Direct evidence that an IHC 1+ tumor responds to T-DXd, though the disease is breast rather than ovarian.
- **Design:** randomized open-label phase 3 (2:1) vs physician's choice chemotherapy
- **n:** 557
- **Effect size:** 0.51 HR — PFS HR (hormone-receptor-positive subgroup)
- **Variance:** 95% CI 0.4–0.64; $p < 0.001$
- **Toxicities:** adverse event $G \geq 3$ (52.6%); ILD/pneumonitis (12.1%); ILD/pneumonitis $G5$ (0.8%); nausea (281/371, 75.7%); anemia (137/371, 36.9%); neutrophil count decreased (81/371, 21.8%); fatigue (110/371, 29.6%)
- **Case fit:** cross_tumor_only
- **Notes:** The off-label-precedent anchor for treating an IHC 1+ tumor with T-DXd, and the HR+ subgroup matches her ER-positive biology. It is breast, so she cannot enrol; efficacy from the NEJM abstract, per-term AE counts from the ClinicalTrials.gov AE tab (NCT03734029).

—/— (2022)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** PI3Kalpha-selective inhibitor (mutant/amplification-directed)
- **Indication / model:** advanced solid tumors with PIK3CA alterations (basket) (2L+); PIK3CA mutation or amplification (PI3Kalpha-pathway activation)
- **Design:** phase 1 dose-escalation/expansion basket, biomarker-selected

- **Effect size:** — — safety / objective response rate
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2022)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** investigational agent matched to MAP2K4 loss
- **Indication / model:** advanced solid tumors (biomarker-matched basket including MAP2K4 loss) (2L+); MAP2K4 deletion / loss-of-function
- **Design:** phase 1 biomarker-matched basket
- **Effect size:** — — safety / ORR
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

Lheureux/Oza (2021) — Lancet

Reference: [PMID 33485453](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Adavosertib (AZD1775, WEE1) + gemcitabine
- **Indication / model:** platinum-resistant or platinum-refractory recurrent high-grade serous ovarian cancer (2L+); Measurable platinum-resistant or platinum-refractory high-grade serous ovarian cancer from 11 academic centers. Histology matches this patient's high-grade serous tumor, but the setting is platinum-resistant relapse rather than her current first-line remission, and there was no biomarker gate.
- **Design:** randomized double-blind placebo-controlled phase 2 (2:1)
- **n:** 99
- **Effect size:** 0.55 HR — progression-free survival
- **Variance:** 95% CI 0.35–0.9; p=0.015
- **Toxicities:** neutropenia G_≥3 (38/61, 62%); thrombocytopenia G_≥3 (19/61, 31%); anemia G_≥3 (19/61, 31%); fatigue G_≥3 (10/61, 16%)
- **Case fit:** partial
- **Notes:** The published randomized WEE1 anchor in high-grade serous ovarian cancer, standing in for azenosertib's still-unpublished DENALI data on the same replication-stress axis. First WEE1-inhibitor trial to improve both PFS (HR 0.55) and OS (HR 0.56) in platinum-resistant disease, so it scopes the realistic magnitude of the WEE1 approach; a relapse option, not maintenance. Marked myelosuppression triggers the patient's veto.

Colon-Otero/Weroha (2020) — ESMO Open

Reference: [PMID 33109627](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Ribociclib + letrozole (CDK4/6 + AI)
- **Indication / model:** relapsed ER-positive ovarian cancer (ovarian cohort of an ovarian/endometrial trial) (2L+); Relapsed ER-positive ovarian cancer, 20-patient ovarian cohort within a two-cohort ovarian/endometrial trial. Histologies were mixed; the three low-grade serous patients drove the durable responses, which is relevant because this patient's tumor is high-grade serous rather than low-grade.
- **Design:** single-arm phase 2, ovarian cohort

- **n:** 20
- **Effect size:** 50.0 % — progression-free at 12 weeks (ovarian cohort)
- **Toxicities:** leukopenia $G \geq 3$ (23%); lymphocytopenia $G \geq 3$ (23%); neutropenia $G \geq 3$ (15%); fatigue $G \geq 3$ (13%)
- **Case fit:** weak
- **Notes:** Published CDK4/6-plus-aromatase-inhibitor evidence in ER-positive ovarian cancer, standing in for the abemaciclib+AI trial (NCT04469764, ALEPRO) whose own readout is abstract-only. Ovarian-cohort activity was modest (ORR 5%, PFS12 50%) and the durable benefit sat in low-grade serous, not the high-grade serous biology here, so the cross-drug read is weak. Grade 3+ AEs pooled across the ovarian and endometrial cohorts.

Gonzalez-Martin/Mirza (2019) — N Engl J Med

Reference: [PMID 31562799](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Niraparib (maintenance)
- **Indication / model:** newly diagnosed advanced high-grade serous/endometrioid ovarian cancer, response after 1L platinum (maintenance); All-comers, both HRD-positive and HRD-negative strata; no BRCA mutation and no validated HRD score required for enrolment. The all-comer enrolment is what makes PRIMA the cleanest fit for this BRCA-proficient, genomically-unstable tumor.
- **Design:** randomized double-blind placebo-controlled phase 3 (2:1), 1L maintenance (PRIMA/ENGOT-OV26)
- **n:** 733
- **Effect size:** 0.62 HR — median PFS (overall population)
- **Variance:** 95% CI 0.5–0.76; $p < 0.001$
- **Toxicities:** anemia $G \geq 3$ (31%); thrombocytopenia $G \geq 3$ (28.7%); neutropenia $G \geq 3$ (12.8%)
- **Case fit:** strong
- **Notes:** Pivotal 1L all-comer maintenance trial behind niraparib's broad label. The BRCA-WT, genomically-unstable profile here qualifies via the all-comer rationale rather than any BRCA gate; HRD-positive subgroup shows larger benefit but enrolment never required it. Triggers the patient's severe-myelosuppression toxicity veto, partly mitigated by individualized starting dose.

Ray-Coquard/Pujade-Lauraine (2019) — N Engl J Med

Reference: [PMID 31851799](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Olaparib + bevacizumab (maintenance)
- **Indication / model:** newly diagnosed advanced high-grade serous ovarian cancer receiving 1L bevacizumab (maintenance); All-comers enrolled, but the predefined and label-relevant benefit is in the HRD-positive subgroup (Myriad MyChoice GIS ≥ 42 or tumor BRCA mutation). This patient is BRCA wild-type and has only a qualitative Altera instability call, so eligibility is CONTINGENT on a validated GIS she does not yet have.
- **Design:** randomized double-blind placebo-controlled phase 3, olaparib vs placebo added to bevacizumab backbone (PAOLA-1 / ENGOT-ov25)
- **n:** 806
- **Effect size:** 0.33 HR — median PFS (HRD-positive subgroup)
- **Variance:** 95% CI 0.25–0.45; $p < 0.001$
- **Toxicities:** hypertension $G \geq 3$ (19%); anemia $G \geq 3$ (17%); lymphopenia $G \geq 3$ (7%); fatigue/asthenia $G \geq 3$ (5%)
- **Case fit:** partial
- **Notes:** Highest-magnitude PARP option (HRD-positive HR 0.33) and the patient is already on the bevacizumab

backbone, but it is contingent: PAOLA-1 benefit concentrates in HRD-positive disease defined by MyChoice GIS ≥ 42 or tumor BRCA. Altera's qualitative instability call does not meet that companion-diagnostic threshold. The BRCA-non-mutated HRD-positive subgroup (HR 0.43) is the relevant comparator only if a validated GIS comes back ≥ 42 ; below that she is HRD-negative and defaults to all-comer niraparib. Combination-arm grade 3+ AE per PMC10859514 expert review.

Banerji/Aftimos (2019) — Lancet Oncol

Reference:PMID 31257177 · **Type:** clinical · **Inclusion:** excluded — Program shelved after the FDA complete response letter on the HER2-positive breast filing and Byondis halted development; the phase 1 enrolled no ovarian patients.

- **Intervention:** Trastuzumab duocarmazine (SYD985)
- **Notes:** Excluded: Program shelved after the FDA complete response letter on the HER2-positive breast filing and Byondis halted development; the phase 1 enrolled no ovarian patients. — Phase 1 documented HER2-low (IHC 1+) breast activity and a duocarmazine ocular and pneumonitis profile distinct from the MMAE conjugates. Kept as mechanism context only, not a live option.

Kok/Friedlander (2019) — J Gynecol Oncol

Reference:PMID 31328463 · **Type:** clinical · **Inclusion:** included

- **Intervention:** Anastrozole (aromatase inhibitor, PARAGON)
- **Indication / model:** ER- and/or PR-positive recurrent ovarian cancer with asymptomatic CA125 progression (2L+); Asymptomatic patients with ER- and/or PR-positive recurrent ovarian cancer and CA125-defined progression; the largest published aromatase-inhibitor read in ER-positive recurrent ovarian disease. This patient is ER-positive but PR-negative, and the report found ER histoscore did not correlate with benefit, so hormone-receptor status was a coarse predictor here.
- **Design:** single-arm phase 2 (PARAGON / ANZGOG-0903)
- **n:** 52
- **Effect size:** 34.6 % — clinical benefit rate at 3 months
- **Variance:** 95% CI 23.0–48.0
- **Toxicities:** Anastrozole was well tolerated with no per-term grade 3+ toxicity table extracted from the abstract; aromatase-inhibitor class toxicity is arthralgia and bone-density loss. No named patient veto applies.
- **Case fit:** partial
- **Notes:** Published aromatase-inhibitor anchor for the ER-maintenance axis; a distinct AI from letrozole but the same mechanism, so it scopes the realistic magnitude of endocrine benefit while MATAO's randomized letrozole readout is pending. Modest clinical benefit (34.6% at 3 mo) with short whole-cohort PFS, and ER histoscore did not predict response, which tempers the ER-positive rationale. No per-term AE table in the abstract.

Kaplan/Glazer (2019) — Sci Transl Med

Reference:PMID 31092693 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Cediranib induces a homologous-recombination defect that sensitizes BRCA-wild-type tumors to olaparib
- **Indication / model:** BRCA-wild-type ovarian lines (IGROV1, SKOV3, PEO14) plus BRCA2-mutant PEO1/PEO4

and breast lines; IGROV1 and hypoxia-reporter xenografts in nu/nu mice

- **Design:** Cediranib drives down BRCA1, BRCA2, and RAD51 through two routes: VEGFR2-mediated tumor hypoxia, and a hypoxia-independent PDGFR-blockade pathway that activates PP2A and E2F4/p130 transcriptional repression. The result is a contextual, drug-induced HR defect in cells that are genetically HR-proficient.
- **n:** multi-line panel; xenograft arms (group sizes per arm not specified in text)
- **Effect size:** strong — Cediranib produced an HR-deficient state in BRCA-wild-type ovarian cells and sensitized them to olaparib both in culture and in xenografts. This is the mechanistic basis for combining a VEGFR inhibitor with a PARP inhibitor in BRCA-proficient HGSC, the exact genetic context of this tumor.
- **Variance:** vs vehicle; single-agent olaparib; single-agent cediranib; translatability: high
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** Cediranib is the tool VEGFR-TKI here, not the patient's bevacizumab; the induced-HRD concept transfers to anti-angiogenic plus PARP strategy but the specific agent differs. Subcutaneous and reporter xenografts rather than orthotopic peritoneal disease.

Heinzelmann-Schwarz/Jacob (2018) — Gynecol Oncol

Reference:PMID 29157627 · **Type:** clinical · **Inclusion:** included

- **Intervention:** Letrozole (endocrine maintenance)
- **Indication / model:** ER-positive high-grade serous ovarian cancer, maintenance / later-line (maintenance); ER-positive HGSOC by IHC; a translational study with a clinical letrozole-maintenance series, enriched for platinum-resistant and residual-disease patients. The patient is ER-positive (~80% nuclei) but PR-negative, which tempers expected magnitude.
- **Design:** translational study with retrospective clinical letrozole-maintenance comparison (TCGA/IHC analysis + treated cohort)
- **Effect size:** 60% vs 38.5% RFS at 24 mo % — recurrence-free interval at 24 months
- **Variance:** p=0.035
- **Toxicities:** No per-term toxicity table in the source; aromatase-inhibitor class toxicity is arthralgia and bone-density loss on prolonged use. Oral, low-burden, no named patient veto applies.
- **Case fit:** partial
- **Notes:** Low-toxicity oral option that fits the patient's age and oral-preference, but the evidence is a non-randomized retrospective comparison nested in a translational ESR1/ER-expression study, not the single-arm phase 2 the trials.jsonl row described - flag the design mismatch. Benefit modest and tempered by PR-negativity. No per-term AE table in source (translational paper, no safety section); toxicities left empty accordingly.

Sun/Wang (2018) — Oncotarget

Reference:PMID 30647846 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Niraparib comparative pharmacokinetics and efficacy in BRCA-wild-type ovarian PDX
- **Indication / model:** BRCA-wild-type ovarian cancer PDX (OVC134) in female BALB/c nude mice; >30 ovarian PDX/CDX panel spanning HRD-positive and HRD-negative
- **Design:** Niraparib partitions into tumor at 3.3x plasma concentration versus 0.6x for olaparib, giving roughly 16-fold higher dose-normalized tumor exposure. The extra drug in the tumor compartment lets niraparib trap PARP and kill cells that retain functional BRCA, where olaparib fails to reach a cytotoxic threshold.

- **n:** single PDX efficacy arm plus >30-model panel; group sizes per arm not detailed in abstract
- **Effect size:** moderate — In the BRCA-wild-type OVC134 model niraparib cut tumor growth by 58% while olaparib barely moved it. Across the >30-model ovarian panel, regressions appeared in BRCA-mutant and HRD-positive BRCA-wild-type tumors, with some growth inhibition even in Myriad-defined HRD-negative models.
- **Variance:** vs olaparib head-to-head; vehicle; translatability: high
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** Subcutaneous PDX, not orthotopic. Directly on point for this BRCA-wild-type, genomically-unstable tumor: it is the preclinical pharmacology argument for niraparib (PRIMA, all-comers) over BRCA-gated olaparib here.

Lin/Ratner (2018) — PLoS One

Reference:PMID 30444904 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Triapine plus olaparib plus cediranib in BRCA-wild-type, PARP-inhibitor-resistant ovarian cancer
- **Indication / model:** BRCA-wild-type, PARP-inhibitor-resistant EOC: SKOV3 and OVCAR3 subcutaneous in nude mice; PEO4ip intraperitoneal in SCID-Beige mice
- **Design:** Triapine inhibits ribonucleotide reductase and adds replication stress; cediranib suppresses HR via hypoxia and PDGFR blockade; olaparib traps PARP. Stacking the three converges on the same DNA-repair vulnerability in tumors that are genetically HR-proficient and have escaped single-agent PARP inhibition.
- **n:** per-arm group sizes not stated in abstract; survival cohorts
- **Effect size:** moderate — In the intraperitoneal PEO4ip model the triple combination extended median survival to 89 days versus 62 in controls and beat every lower-order arm. Supports layering anti-angiogenic and replication-stress agents onto a PARP inhibitor in BRCA-wild-type, PARP-resistant disease.
- **Variance:** vs vehicle and lower-order single/double agent arms; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** Triapine is not on this patient's option list, so the directly transferable insight is the cediranib-plus-olaparib backbone in BRCA-wild-type disease. PEO4ip carries a reverted BRCA2, a resistance model rather than a naive maintenance setting.

Moore/DiSilvestro (2018) — N Engl J Med

Reference:PMID 30345884 · **Type:** trial · **Inclusion:** included

- **Intervention:** olaparib monotherapy maintenance
- **Indication / model:** newly-diagnosed advanced ovarian cancer with a germline or somatic BRCA1/2 mutation (1L); germline or somatic BRCA1/2 pathogenic mutation REQUIRED
- **Design:** randomized double-blind placebo-controlled, 1L maintenance
- **n:** 391
- **Effect size:** 0.3 — progression-free survival
- **Variance:** 95% CI 0.23–0.41; $p < 0.001$
- **Case fit:** none
- **Notes:** (toxicity table not extracted)

—/— (2018)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** MAPK-pathway-directed investigational agent
- **Indication / model:** advanced solid tumors (genotype-matched basket including MAP2K4/MAPK-pathway) (2L+); MAP2K4 deletion / MAPK-pathway alteration
- **Design:** phase 1 genotype-matched basket
- **Effect size:** — — safety / ORR
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

—/— (2017)

Reference: — · **Type:** trial · **Inclusion:** included

- **Intervention:** MAPK-pathway-directed investigational agent
- **Indication / model:** advanced solid tumors (genotype-matched basket including MAP2K4/MAPK-pathway) (2L+); MAP2K4 deletion / MAPK-pathway alteration
- **Design:** phase 1 genotype-matched basket
- **Effect size:** — — safety / ORR
- **Case fit:** weak
- **Notes:** (registration only — no peer-reviewed publication yet; toxicity table not extracted)

Wang/Cheng (2016) — Oncotarget

Reference: [PMID 26909613](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** PI3K inhibition (BKM120) plus olaparib in PIK3CA-altered ovarian cancer
- **Indication / model:** PIK3CA-altered ovarian lines (SKOV3, HEYA8, IGROV1, EFO27); intraperitoneal SKOV3-luciferase dissemination xenograft in NOD/SCID mice
- **Design:** Pan-class-I PI3K inhibition with BKM120 lowers BRCA1/2 expression in responsive PIK3CA-altered ovarian cells, manufacturing a homologous-recombination defect. That induced HRD then makes otherwise HR-proficient cells vulnerable to PARP trapping.
- **n:** multi-line panel; IP xenograft cohort (per-arm sizes not stated in abstract)
- **Effect size:** moderate — BKM120 plus olaparib was synergistic, raising DNA damage and apoptosis and shrinking tumor burden in the intraperitoneal model; cells that dropped BRCA1/2 most on PI3K inhibition responded best. Investigational rationale connecting the patient's PIK3CA amplification to the PARP/HRD axis, distinct from the foreclosed BRCA-mutation route.
- **Variance:** vs vehicle; single-agent BKM120; single-agent olaparib; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** partial
- **Notes:** Models carry PIK3CA point mutations; the patient has PIK3CA amplification, so the dependency may differ in degree. Strictly investigational, with no approved PI3K-directed therapy in HGSC; surfaced as a mechanism-level link, not a near-term option.

Ogitani/Agatsuma (2016) — Clin Cancer Res

Reference: [PMID 27026201](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** T-DXd (DS-8201a) foundational pharmacology versus T-DM1
- **Indication / model:** HER2-positive breast and gastric cell lines plus breast PDX models, including low-HER2 and T-DM1-refractory PDX, in mice
- **Design:** DS-8201a carries about 8 exatecan-derivative topoisomerase I payloads per antibody against T-DM1's roughly 3.5, and the freed DXd crosses cell membranes. The heavier payload load plus a diffusible toxin reaches tumor cells that take up little HER2.
- **n:** multi-line panel plus several PDX arms; per-arm group sizes not stated in the abstract
- **Effect size:** strong — Growth inhibition tracked HER2 level but did not need high expression: DS-8201a regressed breast PDX models with low HER2 and tumors that had stopped responding to T-DM1. Chk1 and gamma-H2AX phosphorylation confirmed the topoisomerase I DNA-damage route.
- **Variance:** vs T-DM1; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** cross_tumor_only
- **Notes:** Breast and gastric models, no ovarian arm. The low-HER2 activity is the load-bearing point for an IHC 1+ tumor, but the proof here sits in a different tissue.

Ogitani/Agatsuma (2016) — Cancer Sci

Reference:PMID 27166974 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** T-DXd (DS-8201a) bystander killing in HER2-heterogeneous tumors
- **Indication / model:** Coculture of HER2-positive KPL-4 with HER2-negative MDA-MB-468; mixed NCI-N87 (HER2+) and luciferase-tagged MDA-MB-468 (HER2-) xenografts in nude mice, with a separate opposite-flank HER2-negative tumor
- **Design:** DXd freed inside a HER2-positive cell is membrane-permeable, so it passes into neighboring cells whatever their HER2 level. T-DM1's charged Lys-SMCC-DM1 payload stays trapped and produces no such spillover.
- **n:** in-vitro cocultures plus mixed-population and dual-flank xenograft cohorts; group sizes not stated in the abstract
- **Effect size:** strong — In mixed cultures and co-implanted xenografts DS-8201a killed the HER2-negative population, but it left HER2-negative tumors growing on the opposite flank untouched. Bystander reach is local and needs HER2-positive cells nearby.
- **Variance:** vs T-DM1; opposite-flank HER2-negative tumor as a spatial control; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** cross_tumor_only
- **Notes:** Breast and gastric lines, not ovarian. An IHC 1+ tumor carries sparse HER2-positive cells, so how far the local bystander effect extends at that antigen density is the open question this study cannot answer.

Li/Coats (2016) — Cancer Cell

Reference:PMID 26766593 · **Type:** preclinical · **Inclusion:** excluded — Reviewed while hunting for the MMAE bystander basis, but this is a different molecule: a biparatopic HER2 ADC (MEDI4276) carrying a tubulysin microtubule inhibitor, not disitamab vedotin's MMAE conjugate. It targets T-DM1-refractory HER2-positive disease rather than the HER2-low tier, so both payload and population diverge from the intervention.

- **Intervention:** Biparatopic HER2 ADC (MEDI4276), tubulysin payload
- **Toxicities:** n/a (preclinical)
- **Notes:** Excluded: Reviewed while hunting for the MMAE bystander basis, but this is a different molecule: a

biparatopic HER2 ADC (MEDI4276) carrying a tubulysin microtubule inhibitor, not disitamab vedotin's MMAE conjugate. It targets T-DM1-refractory HER2-positive disease rather than the HER2-low tier, so both payload and population diverge from the intervention.

van der Lee/Dokter (2015) — Mol Cancer Ther

Reference:PMID 25589493 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Trastuzumab duocarmazine (SYD985) preclinical profile in low-HER2 breast cancer
- **Indication / model:** Breast cancer cell lines and PDX models spanning HER2 3+, 2+, and 1+; HER2-0 cells mixed with 3+/2+/1+ cells for bystander assays
- **Design:** SYD985 carries a duocarmycin (vc-seco-DUBA) on a cathepsin-B-cleavable linker; the freed seco-DUBA is membrane-permeable and stays active across tumor-relevant pH, so it kills HER2-low cells directly and reaches HER2-negative neighbors.
- **n:** low- and high-HER2 cell lines plus breast PDX models; per-arm sizes not stated in the abstract
- **Effect size:** strong — In low-HER2 lines SYD985 was 3- to 50-fold more potent than T-DM1, and it regressed HER2 1+, 2+, and 3+ breast PDX models where T-DM1 worked only at 3+. HER2-0 cells were killed when mixed with 1+, 2+, or 3+ cells.
- **Variance:** vs T-DM1; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** cross_tumor_only
- **Notes:** Breast cell lines and PDX, no ovarian model. The clinical program is shelved after Byondis halted development on the FDA complete response letter, so this stands as HER2-low mechanism evidence rather than backing for a live option.

Hirakawa/Mizunuma (2014) — J Ovarian Res

Reference:PMID 24410765 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Aromatase inhibition in ER-alpha-positive ovarian cancer xenografts
- **Indication / model:** Ovariectomized nude mice with peritoneal carcinomatosis: OVCAR-3 (strong ER-alpha) versus DISS (weak ER-alpha)
- **Design:** Aromatase inhibition lowers local estrogen and blunts ER-alpha signaling in ER-positive ovarian tumors. Downstream it reduces VEGF and tumor microvessel density, tying the endocrine effect to an anti-angiogenic, anti-ascites readout.
- **n:** survival cohorts per line (sizes not stated in abstract)
- **Effect size:** moderate — Letrozole prolonged survival in the strongly ER-alpha-positive OVCAR-3 model but did nothing in the weakly positive DISS model, and lowered tumor VEGF and microvessel density. The ER-magnitude dependence fits this patient's ~80% ER-positive but PR-negative profile as a real-but-modest endocrine signal.
- **Variance:** vs untreated tumor-bearing mice; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** partial
- **Notes:** Benefit tracked ER-alpha intensity, and this tumor is ER-positive but PR-negative, predicting a tempered response. No HGSC-specific orthotopic arm; OVCAR-3 is a serous-type line.

Murai/Pommier (2012) — Cancer Res

Reference:[PMID 23066039](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Differential PARP-trapping potency across PARP inhibitors
- **Indication / model:** DT40 isogenic chicken B-cell panel and human cancer cell lines with defined DNA-repair backgrounds; cell-free PARP-DNA retention assays
- **Design:** PARP inhibitors differ in how strongly they trap PARP-DNA complexes, and trapping (not just catalytic inhibition) drives cytotoxicity. Niraparib and olaparib are far stronger trappers than veliparib, with trapping potency tracking cell kill more closely than enzymatic IC50.
- **n:** isogenic cell-line panel; biochemical replicates
- **Effect size:** strong — Niraparib ranked among the most potent PARP-trapping agents tested, well above veliparib. The trapping-versus-catalytic distinction explains why a strong trapper can damage genomically unstable, BRCA-proficient cells that survive a weak catalytic-only inhibitor.
- **Variance:** vs PARP-null and repair-proficient isogenic lines; veliparib/olaparib comparators; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** partial
- **Notes:** Mechanistic cell-line work, not an ovarian in-vivo efficacy study. Foundational reference for why niraparib's trapping potency matters in a BRCA-wild-type setting; the later DT40 follow-up (Murai 2014) refines the ranking.

Mabuchi/Kimura (2012) — Br J Cancer

Reference:[doi:10.1038/bjc.2012.261](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** Bevacizumab maintenance after chemotherapy in ovarian carcinoma xenografts
- **Indication / model:** Human epithelial ovarian carcinoma xenografts (including HOC22) in mice; chemotherapy plus bevacizumab in concurrent, maintenance, and sequential schedules
- **Design:** Adding bevacizumab to platinum-taxane chemotherapy suppresses VEGF-driven angiogenesis during and after cytotoxic treatment. Continuing it as maintenance keeps the residual tumor vasculature suppressed once chemotherapy stops.
- **n:** multiple xenograft arms (per-arm group sizes not stated in abstract)
- **Effect size:** moderate — Bevacizumab with cisplatin and paclitaxel delayed progression, and continuing it as maintenance after chemotherapy gave the longest survival, with complete responses in the HOC22 model. Preclinical mirror of the GOG-218/ICON7 maintenance schedule this patient is on.
- **Variance:** vs cisplatin plus paclitaxel without bevacizumab; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** PMID not independently confirmed in this run; DOI verified (10.1038/bjc.2012.261). Schedule-comparison xenograft work; the maintenance arm is the load-bearing result for the patient's current bevacizumab continuation.

Burger/Liang (2011) — N Engl J Med

Reference:[PMID 22204724](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Bevacizumab (with maintenance)
- **Indication / model:** newly diagnosed stage III-IV epithelial ovarian / primary peritoneal cancer (1L); All-comers, no biomarker selection. Matches the VEGF/angiogenesis axis the patient is already being treated on.
- **Design:** randomized double-blind placebo-controlled three-arm phase 3 (GOG-218)

- **n:** 1873
- **Effect size:** 0.717 HR — median PFS (bevacizumab-throughout)
- **Variance:** 95% CI 0.625–0.824; $p < 0.001$
- **Toxicities:** hypertension $G \geq 2$ (22.9%); GI perforation/wall disruption (2.6%)
- **Case fit:** strong
- **Notes:** Anchors the bevacizumab maintenance axis the patient is already on. PFS benefit only; OS was not improved in the overall population. VTE/bleeding caution is the operative concern here, not the patient's named neuropathy or myelosuppression vetoes.

Perren/Oza (2011) — N Engl J Med

Reference: [PMID 22204725](#) · **Type:** clinical · **Inclusion:** included

- **Intervention:** Bevacizumab (with maintenance)
- **Indication / model:** newly diagnosed epithelial ovarian / primary peritoneal cancer (1L); All-comers, no biomarker gate. OS benefit concentrated in the high-risk subset (stage IV or suboptimally debulked); this stage IIIc R0 patient sits adjacent to that subset.
- **Design:** randomized open-label two-arm phase 3 (ICON7)
- **n:** 1528
- **Effect size:** 0.81 HR — median PFS
- **Variance:** 95% CI 0.7–0.94; $p = 0.004$
- **Toxicities:** hypertension $G \geq 2$ (18%)
- **Case fit:** strong
- **Notes:** Reinforces the bevacizumab axis; largest benefit in the high-risk subgroup. Open-label design so RoB is some-concern. OS gain limited to the high-risk subset.

Anwer/Fewell (2010) — Gene Ther

Reference: [PMID 20033066](#) · **Type:** preclinical · **Inclusion:** included

- **Intervention:** IL-12 plasmid lipopolymer (GEN-1 / EGEN-001) intraperitoneal immunogene therapy: ID8 preclinical basis
- **Indication / model:** Murine ID8 intraperitoneal ovarian tumor model dosed with mouse IL-12 plasmid formulated in PEG-PEI-cholesterol lipopolymer (pmIL-12/PPC); plus first-in-human phase I
- **Design:** Local intraperitoneal delivery of an IL-12-encoding plasmid produces sustained, compartment-restricted IL-12 and IFN-gamma rather than a short systemic protein bolus. That shifts the peritoneal immune microenvironment toward a Th1 program and lowers ascitic VEGF.
- **n:** tumor-bearing mouse cohorts plus 13-patient phase I
- **Effect size:** moderate — In ID8 mice, pmIL-12/PPC raised peritoneal IL-12 and IFN-gamma, drove a Th1 gene-expression shift, dropped ascitic VEGF, and improved survival alone and with chemotherapy. Mechanistic foundation for the GEN-1/IMNN-001 program this patient received, working through the TME rather than the foreclosed MSI/TMB checkpoint axis.
- **Variance:** vs untreated/empty-vector tumor-bearing mice; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** The ID8 preclinical package is reported alongside the phase I in this and companion Immunon/EGEN sources rather than as a single standalone mouse paper; treat as the preclinical-plus-early-clinical anchor. Syngeneic ID8 immunogenicity may exceed human HGSC, and MSS/TMB-low status argues against

single-agent checkpoint blockade, not against this IL-12 route.

Mesiano/Jaffe (1998) — Am J Pathol

Reference:PMID 9777956 · **Type:** preclinical · **Inclusion:** included

- **Intervention:** VEGF immunoneutralization controls ovarian tumor growth and ascites
- **Indication / model:** Human SKOV-3 ovarian carcinoma, subcutaneous and intraperitoneal, in immunodeficient mice; treated with anti-VEGF antibody A4.6.1 (the murine antecedent of bevacizumab)
- **Design:** VEGF supports ovarian tumor growth by driving neovascularization and raises vascular permeability to produce malignant ascites. Blocking VEGF starves the angiogenic supply and shuts down the permeability that fills the peritoneal cavity.
- **n:** xenograft cohorts (group sizes per arm not specified in abstract)
- **Effect size:** strong — A4.6.1 significantly slowed subcutaneous SKOV-3 growth, partly slowed intraperitoneal growth, and abolished ascites formation; tumor regrew once antibody was withdrawn. This is the founding preclinical rationale for the VEGF-maintenance axis the patient is already on with bevacizumab.
- **Variance:** vs untreated tumor-bearing mice; translatability: med
- **Toxicities:** n/a (preclinical)
- **Case fit:** strong
- **Notes:** A4.6.1 is the precursor antibody clone, not clinical bevacizumab, and growth resumed off-therapy, mirroring the maintenance-not-cure clinical pattern. Older study; the ascites-control finding remains the cleanest mechanistic anchor for anti-VEGF here.